



OFFICIAL

NMHS MHS POLICY

Infection Prevention and Control (IPC) - Invasive Devices

Scope (Audience)	All NMHS Mental Health Services (MHS) Staff
Scope (Area)	All NMHS MHS Areas
Summary	To prevent the acquisition and transmission of micro-organisms associated with invasive devices in NMHS MHS
Risk Rating	Medium
Policy Standards	Clinical Governance (Std 1) Preventing & Controlling Healthcare Associated Infections (Std 3)
Policy Sponsor	Director of Nursing MHSDHS
Policy Contact	Clinical Nurse Manager IPC MHSDHS
Version	3.0
Review Date	28/10/2028

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

To prevent the acquisition and transmission of micro-organisms associated with invasive devices.

2. Background

The presence and use of invasive devices is a recognised risk factor for a healthcare associated infection (HAI) which may cause adverse patient outcomes and prolong the patient's duration of stay in hospital.

Invasive procedures and devices may predispose the patient to infection as they breach the normal defence barriers (e.g. the skin) and act as a portal entry for acquisition and transmission of micro-organisms; therefore, must only be used when clinically indicated.

Raising awareness of the infection risks associated with invasive devices and ensuring devices are managed appropriately and safely, will reduce the risk of any associated infection.

This Policy should be read in conjunction with the [NMHS Infection Prevention and Control Policy SPE 28](#).

3. Risk

The risks and benefits of an invasive device should be carefully considered prior to use, and the device should be removed at the earliest opportunity.

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☒	Other	☐

4. Key Points

- Only staff that have the relevant knowledge, skills and competency, and are working within their scope of practice, can insert, care for and/or remove invasive devices.
- For queries or specialist advice regarding specific invasive devices contact Sir Charles Gairdner Hospital (SCGH).

5. Policy Principles

Staff are required to:

- Ensure that prior to the use of any invasive devices, the device and associated equipment must be thoroughly checked to ensure the sterility and integrity of the items.

- Review the need for continuing treatment with invasive devices daily and remove as soon as they are no longer needed or at the first sign of local site complications, ensuring documentation is maintained.
- Perform a risk assessment and wear Personal Protective Equipment (PPE) as per [MHPHDS IPC Standard & Transmission Based Precautions Policy](#).
- Apply [Aseptic Technique principles](#) and the 5 Moments of Hand Hygiene when inserting, removing or caring for invasive devices.
- Where practicable, involve the patient in self-care of their devices by providing ongoing education.
- Manufacturer's instructions for use, cleaning and disinfection must be followed for any reusable medical invasive device.

5.1 NMHS Mental Health Inpatient Facilities

All Mental Health Services will adhere to the principles outlined in the WA Health [MP 0038/16 Insertion and Management of PIVC in Healthcare Facilities Policy](#).

Patients who have been admitted into NMHS MH inpatient facilities with invasive devices must:

- Have a relevant care plan detailing the care of the device and follow the respective guidelines:
 - [SCGH Nursing Practice Guideline: CVC and PICCs](#) to assess for any potential complications (for PIVAS assessment and patient education (refer to *Appendix A*))
 - [Standard AT Uncomplicated Wound](#)
 - [SCGH Nursing Practice Guideline: Bladder Management and Urinary Catheterisation](#) to assess for any potential complications (assessment refer to *Appendix B*).
- **New patients, within 48 hours of admission**, are to undergo relevant pathology collection for potential infections following a risk assessment.

5.2 Subcutaneous (SubCut), Intramuscular (IMI) Injections and Subcutaneous Infusions/Fluid Administration

- Staff to follow the [AT IMI Injection Guideline](#) and dispose of sharps safely at the point of use.
- [Subcutaneous Infusions](#) – follow the respective guidelines for insertion sites / flushing / and reviews in nursing care plans.

5.3 NMHS Mental Health Community Clinics

In general, community patients with Invasive Devices are assessed by their GPs and /or Silver Chain.

Any reports relating to these devices are reviewed by the community care team and placed into the patients notes.

6. Monitoring, Compliance and Evaluation

Compliance with this policy is mandatory.

Monitoring of compliance with this document is to be carried out by MHSDHS IPC Department as required.

This policy will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every three (3) years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

7. Definitions

Terms	Definition
Invasive devices	Are medical devices that breach the body's normal line of defence. Examples of these include: • Intravascular devices e.g. peripheral intravenous catheter [PIVC], central venous catheter, arterial line, haemodialysis catheters, infusaport etc. • Nasogastric Tubes [NGT]. • Tracheostomy. • Urinary Catheters (including suprapubic and urethral catheters) [IDC]. • Intramuscular [IMI] & Subcutaneous [Sub Cut] infusion and / or Sub Cut injection. • Percutaneous feeding tubes [PEG]. • Stomas • Wound • Chest drains

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms.

8. References

- [ACSQHC Clinical Care Standards Management of Peripheral Intravenous Catheters](#)
- [Australian Guidelines for the Prevention and Control of Infection in Healthcare \(2019\) \(see 3.5.2 Invasive medical devices\)](#)
- [National Safety and Quality Health Service Standards – Preventing and Controlling Healthcare Associated Infections](#)

9. Related Documents

Department of Health Government	• Insertion and Management of PIVC in Healthcare Facilities Guideline
NMHS Policy documents	• NMHS Infection Prevention and Control Policy
MHSDHS Policy documents	• MHSDHS IPC Hand Hygiene Policy • MHSDHS IPC Infection Prevention and Control Breach Policy

	• MHSDHS IPC Standard and Transmission Based Precautions Guideline
Standards	• Management of Peripheral Intravenous Catheters Clinical Care Standard

10. Resources

- [ACSQHC Peripheral intravenous catheters: A review of guidelines and research The Australian and New Zealand Urological Nurses Society Inc. \(ANZUNS\). Catheter care guidelines.](#)
- [MHSDHS Aseptic Non-Touch Technique hub page](#)
- [Queensland Government. Queensland Health: Guidelines for Subcutaneous Infusion Device Management in Palliative Care.](#)
- [RBPBG Urethral Catheterisation CPS](#)
- [Royal Perth Bentley Group: CPS for Urethral Catheterisation 2020](#)
- [Royal Perth Bentley Group: Invasive devices Learning Governance Framework 2017](#)
- [Royal Perth Bentley Group: Insertion of a Peripheral Intravenous Cannula \(PIVC\) SDLP 2017](#)

11. Document Control

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and

Document Version Control			
Date	Version	Author	Notes
30/06/2014	V1	Area Director of Nursing	Previously MH guideline – now a MHPHDS Policy
08/04/2022	V2	Area Director of Nursing	Scheduled review, changed from MH Guideline to MHPHDS Policy, updated template, references and hyperlinks, removed microbiology investigations, included mandatory requirements as per the NSQHS standard 3.9.
28/10/2025	V3.0	CNM IPC MHSDHS	Scheduled review, scope of policy changed from MHSDHS to MHS as this is not applicable to Dental Health Services, template update, no content change

community.

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Appendix A: Peripheral Intravenous Assessment Score (PIVAS)

Peripheral Intravenous Assessment Score (PIVAS) Assessment criteria and actions required		
 Assess the need for the PIVC against the <u>DRIP</u> criteria below D Is there a risk of sudden Deterioration? R Is Rehydration needed, are IV fluids required? I Are IV medications required? P Are Procedures requiring IV access planned? If “No” to all the DRIP criteria, discuss alternative management options with the medical team before inserting a cannula. If a PIVC is insitu and if “No” to all DRIP criteria remove the PIVC.		
Assess the PIVC site each time it is accessed and document the PIVAS each shift and as per hospital Guidelines.		
<u>LOOK</u> Observe the PIVC site for erythema, swelling or exudate. Is the dressing clean, dry and intact?	<u>LISTEN</u> Ask the patient or use visual clues. Is there pain or tenderness on infusion, palpitation or movement?	<u>FEEL</u> Palpate the site through the intact dressing. Is there any heat or vessel hardening?
Clinical Signs and Symptoms	PIVAS	Clinical Interventions
NO signs of phlebitis and/ or erythema, swelling, exudate, pain, tenderness and IV site appears healthy.	0	• Replace dressing if not clean, dry, or intact. • Remove if no longer required and continue PIVAS for 48 hours.
ANY evidence of ONE of the following signs / symptoms at or near the IV site - pain, tenderness or erythema.	1	• Remove cannula and continue PIVAS for 48 hours. • Document actions and discuss with medical officer. • Request re-siting if patient still meets DRIP criteria.
TWO of the following signs of Phlebitis are evident at or near the IV site: • Pain • Hardness • Erythema • Purulent discharge • Swelling • Palpable venous cord	2	• Remove cannula. • Document vital signs and discuss with medical officer. • Swab site if discharge present (consider blood cultures). • Re-site if patient still meets DRIP criteria. • Complete CIMS.
Fever $\geq 38^{\circ}$ C not explained by other causes.	 	Obtain 2 sets (4 bottles) of blood cultures from another peripheral vein. Send PIVC tip to pathology.